Medication sourcing opportunity memo

Portfolio analysis | October 31, 2026

Decision summary

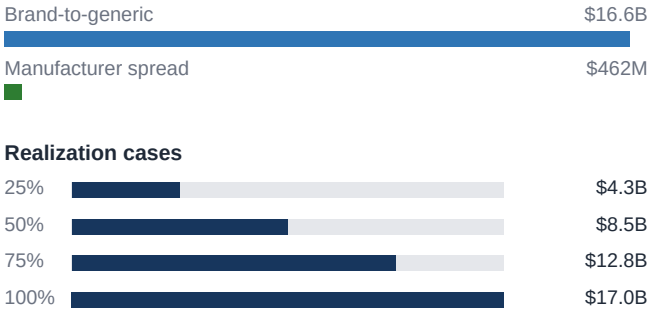
The 50-drug screen identifies a large reference-price gap concentrated in brand-to-generic cases. Use the output to prioritize validation and contracting work, not as a booked savings forecast. CMS reports gross Part D claim cost, while NADAC is a retail pharmacy acquisition-cost reference; the two measures are not interchangeable.

$23.0B
Selected CMS gross spend
2024, 50 unique drug keys

$17.0B
100% reference gap
Unconstrained addressable gap

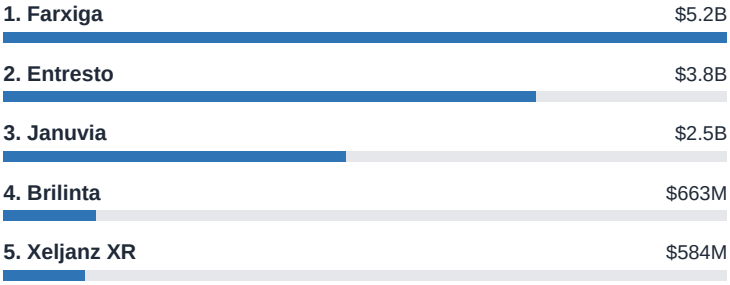
$8.5B
50% realization case
Mechanical sensitivity only

Scenario mix and sensitivity



Realization cases

Largest reference gaps



Method and controls

Selection. 25 brand-to-generic and 25 manufacturer-spread cases, ranked by 2024 CMS gross spend. All 50 drug keys are unique and the two groups do not overlap. **Brand scenario.** Exact normalized product-name prefix, non-OTC, EA-unit NADAC matches; the median NDC-level brand/generic price ratio is applied to CMS gross spend. **Manufacturer scenario.** CMS manufacturer detail must reconcile to the Overall row and current NADAC must show one description; the lowest observed CMS gross-spend-per-unit among meaningful-volume rows is used as a proxy.

Required validation before business use

1) Obtain NDC-level utilization and package/strength mix. 2) Confirm therapeutic and formulary equivalence. 3) Replace gross Part D cost with actual net acquisition and rebate data. 4) Validate manufacturer identity, availability, contract terms, and supply continuity. 5) Re-run on aligned time periods.

Key limitation: CMS has no NDC, strength, form, or route. NADAC has no manufacturer name. Therefore, no row is an exact manufacturer-specific acquisition-cost match.

Sources: CMS Medicare Part D Spending by Drug (2024 release, updated 06/25/2026); NADAC 2026 weekly file through 10/28/2026.

QA: 50 rows | 50 unique | 0 overlap